Nose & Sinus Examination — OSCE Checklist

Deakin MD2 · BB4 · ENT

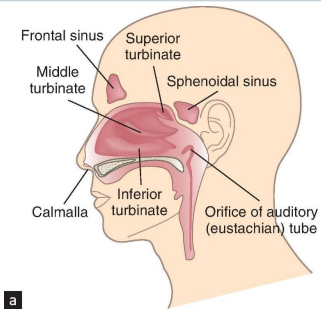
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Inspect externally → Palpate → Airflow → Anterior rhinoscopy → Sinuses**. *Nasal speculum is rarely used outside specialist settings — a pen torch or otoscope is usual.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask about nasal symptoms (blockage, discharge, bleeding, smell); position seated at eye level
☐	Equipment: pen torch, cold metal tongue depressor, nasal speculum or otoscope with wide speculum

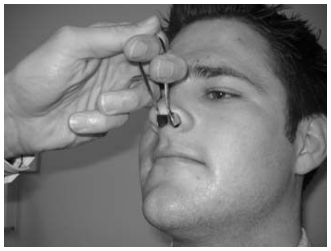
2. Examination

	Step	What to do / say	Abnormal
External inspection			
☐	Front and side	Swelling, deformity, scars or creases, redness / skin disease	Trauma, rhinophyma, BCC
☐	From above	Patient tilts head back; look down the bridge	Deviated nose
☐	Discharge	Rhinorrhoea, crusting; unilateral?	Unilateral offensive in a child = foreign body



Nasal and sinus anatomy

Palpation			
☐	Nasal bones	Gently palpate bridge	Tenderness, crepitus, step = fracture
Airway patency			
☐	Misting test	Cold metal tongue depressor under nose as patient breathes out	Condensation under both nostrils = airflow
☐	Sniff test	Occlude one nostril, ask patient to sniff; repeat	Unilateral obstruction
Anterior rhinoscopy			
☐	Without speculum	Tip nose up with thumb; look into the anterior nares with torch	
☐	With speculum / otoscope	Gently open; look straight back, not up	
☐	Septum	Straight or deviated? Bleeding points, clots, crusting, perforation; septal haematoma	Little's area = common epistaxis site †; haematoma = urgent drainage
☐	Turbinates	Inflammation, swelling, colour	Rhinitis (pale, boggy = allergic †)
☐	Polyps*	Grey, glistening, insensate †	Chronic rhinosinusitis



Nasal speculum examination

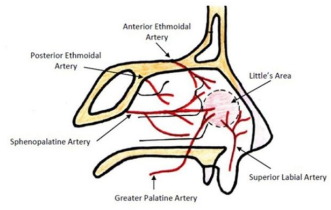


Figure 1. Blood supply to the nasal septum

Blood supply of the septum — Little's area



Nasal polyp

Sinuses			
☐	Palpate	Frontal (above brow) and maxillary (cheek) sinuses	Tenderness = sinusitis
☐	Transilluminate*	Pen torch in a dark room (demonstrated in session)	Reduced = fluid / mass
☐	Note	Ethmoid and sphenoid sinuses cannot be examined physically	

3. Closing

☐	Thank patient; clean equipment; wash hands
☐	Present: "No external deformity; both nasal airways patent; septum midline with no haematoma or perforation; turbinates normal; no sinus tenderness."
☐	Completion: "I would complete the ENT exam, test smell (CN I) if indicated, and consider nasendoscopy."